Example 3 — Acute Decompensated HFpEF With Atrial Fibrillation and Cellulitis

Patient: 81-year-old woman
Admission diagnosis: Acute on chronic HFpEF with volume overload and right lower-extremity cellulitis
Discharge diagnoses:

- Acute on chronic diastolic heart failure
- Atrial fibrillation with rapid ventricular response, resolved
- Right lower-extremity cellulitis
- Hypertension
- Chronic kidney disease stage 3
- Type 2 diabetes
- Chronic lower-extremity edema

Hospital Course

The patient presented with worsening bilateral lower-extremity edema, approximately 10-pound weight gain, orthopnea, and erythema/warmth of the right lower leg. She was found to be in atrial fibrillation with rapid ventricular response.

She received IV diuresis with substantial improvement in edema and dyspnea. Rate control was achieved after adjustment of her beta-blocker regimen. Echocardiogram showed preserved LVEF with findings consistent with HFpEF.

The right lower-extremity cellulitis was treated initially with IV antibiotics, followed by oral therapy after clinical improvement. There was no evidence of abscess or osteomyelitis. Renal function remained stable near baseline.

At discharge, she was breathing comfortably on room air, ambulatory with her usual assistive device, and had improving lower-extremity erythema.

Discharge Medications

New medications

- **Torsemide 20 mg by mouth once daily**, replacing prior intermittent diuretic use.
- **Metoprolol succinate 75 mg once daily** for rate control.
- **Cephalexin 500 mg four times daily for 4 additional days** to complete cellulitis treatment.
- **Empagliflozin 10 mg once daily** for heart failure management.

Continue

- Apixaban 5 mg twice daily, with dose to be taken according to the final medication reconciliation.
- Amlodipine as instructed.
- Atorvastatin nightly.
- Home diabetes regimen as reconciled.

Stopped

- Furosemide — replaced by torsemide.
- Previous short-acting metoprolol regimen — replaced by extended-release formulation.

Discharge Instructions

- Weigh yourself daily.
- Follow a low-sodium diet.
- Elevate the affected leg when resting as instructed.
- Complete the cephalexin course.
- Monitor the cellulitis for decreasing redness, warmth, and swelling.
- Take anticoagulant exactly as prescribed and do not discontinue it without medical advice.
- Follow up with cardiology and primary care.

- Obtain renal function/electrolytes as ordered after the diuretic change.

Return Precautions

Seek urgent evaluation for:

- New chest pain.
- Severe or rapidly worsening shortness of breath.
- Syncope or near-syncope.
- Sustained rapid or irregular heartbeat associated with symptoms.
- Rapidly worsening edema or weight gain.
- Cellulitis spreading beyond the marked area or developing rapidly increasing pain.
- New fever, chills, confusion, or severe weakness.
- Significant bleeding while taking anticoagulation.

Nursing Discharge Teaching / What Nursing Typically Relays

Nursing commonly reviews:

- The **old diuretic versus the new diuretic**, making clear that the patient should not take both unless specifically instructed.
- Expected increased urination with torsemide.
- Daily-weight technique and symptom monitoring.
- Antibiotic schedule and completion.
- Skin/leg monitoring, including worsening redness, swelling, drainage, or fever.
- Anticoagulation precautions.
- Fall precautions if the patient has experienced dizziness or blood-pressure changes.
- Follow-up appointments and laboratory monitoring.
- Teach-back of the medication changes before leaving the hospital.